DRUG PIPELINE WATCH LIST · 2026-2027

The Pipeline That Will Hit Your Plan.

Five drug categories most likely to affect self-funded pharmacy spend in the next 18 months. Cost framing, plan-impact summary, and prep checklist for each. The pipeline is known today; the question is whether you are.

01 The Five Categories

Each panel names a category, the cost framing that makes it consequential for self-funded plans, and four prep steps to take before the first claim arrives. Check off each step as it is completed, or use the panel as a quarterly conversation prompt with your broker.

GLP-1 and Obesity / Metabolic

Indication expansion · 4+ new conditions

Additional GLP-1 formulations advancing, including oral versions and combination therapies. Indications expanding beyond diabetes and obesity into cardiovascular risk reduction, NASH/MASH, sleep apnea, and kidney disease. Biosimilar competition emerging for early GLP-1 products.

- [ ] Review current GLP-1 coverage policy and medical necessity criteria
- [ ] Evaluate step therapy protocols across the class
- [ ] Model financial impact of expanded indications on your population
- [ ] Discuss biosimilar substitution strategy with your PBM before launch

Gene and Cell Therapy

$500K - $3.5M / treatment

Gene therapies approaching approval for hemophilia, sickle cell, and inherited retinal conditions. CAR-T cell therapy expanding to earlier lines of cancer treatment. CRISPR-based gene editing entering clinical use. A single claim can exceed specific stop-loss attachment.

- [ ] Confirm stop-loss policy addresses gene and cell therapies explicitly
- [ ] Coordinate with PBM on adjudication path: pharmacy vs medical benefit
- [ ] Develop a coverage position before the first claim arrives
- [ ] Evaluate outcomes-based payment models tied to treatment success

Alzheimer's Disease

$26K+ / year + infusion fees

Additional amyloid-targeting therapies following lecanemab. Companion diagnostic requirements limit eligible population. Many of these therapies are medical benefit, not pharmacy. If the two benefits are not coordinated, coverage criteria can fall through the gap between them.

- [ ] Determine how your plan will handle anti-amyloid therapies
- [ ] Coordinate coverage criteria between medical and pharmacy benefits
- [ ] Evaluate site-of-care requirements and infusion management
- [ ] Review stop-loss coverage for this category specifically

Oncology

Treatment durations extending; oral oncology shifting to pharmacy

Targeted therapy combinations increasing complexity. Antibody-drug conjugates expanding across tumor types. Bispecifics entering standard of care. Combination regimens and longer durations create ongoing cost pressure; oral oncology shifts costs from medical to pharmacy benefit.

- [ ] Review specialty pharmacy management for oncology
- [ ] Monitor oral oncology as a percentage of total pharmacy spend
- [ ] Evaluate site-of-care steering for infused therapies
- [ ] Ensure clinical pathways align with evidence-based guidelines

Rare Disease

Single claim can exceed specific stop-loss

Orphan drug approvals continue at an accelerating pace. Gene therapies for rare conditions carry extreme price points; enzyme replacement therapies face limited competition. Populations are small; you may never see a claim. If you do and the plan has no position, exposure is severe.

- Review your plan's rare disease claims over the past 24 months
- Identify high-cost rare disease therapies; confirm stop-loss coordination
- Evaluate specialty pharmacy management for rare disease categories
- Track pipeline for therapies that may affect your specific population

Illustrative pipeline categories for educational purposes. Specific drugs, indications, and price points evolve quarterly. Confirm current FDA approval status and PDUFA dates before acting.

03 Quarterly Pipeline Review Process

Pipeline monitoring does not require a dedicated team or expensive tools. It requires a consistent quarterly process. A few hours, four times a year. Run these four steps each quarter and treat the outputs as standing inputs to your renewal preparation.

STEP 1 · EACH QUARTER

Pull FDA PDUFA dates for the upcoming quarter.

PDUFA target action dates are publicly available and tell you when approval decisions are expected. Filter to therapies that match your plan's claim categories, age demographics, and high-cost claimant profile.

STEP 2 · EACH QUARTER

Request a pipeline briefing from your PBM.

Most large PBMs offer this service but rarely push it proactively. If your PBM has not been providing pipeline intelligence, request it. Your broker or consultant can help structure the request as a standing quarterly deliverable, not a custom ask.

STEP 3 · EACH QUARTER

Cross-reference pipeline drugs against your plan's utilization.

A gene therapy for a condition no one in your plan has is lower priority than a new indication for a drug class already generating significant spend. Map the pipeline against your specific population and disease prevalence, not the broad U.S. market.

STEP 4 · EACH QUARTER

Update coverage positions and stop-loss provisions.

For any pipeline category that intersects your plan, draft or update the coverage position now. Confirm stop-loss explicitly addresses the category. The drugs that will affect your plan next year are knowable today; preparation is the difference between planning and shock.

04 What to Require From Your PBM

Your PBM contract should include language that supports proactive pipeline management. PBMs already track pipeline drugs extensively and negotiate manufacturer contracts months before approval. The communication gap is not an information gap; it is a contract gap. Close it with these four asks at the next renewal redline.

§ **Advance notification of pipeline approvals relevant to your plan**
Quarterly written briefings on pipeline drugs that intersect your therapeutic spend categories, with manufacturer contract status and projected formulary position. Standing deliverable, not a custom request.

§ **Default coverage position with employer override window**
Understand the PBM's default coverage position for newly approved drugs and confirm a defined window during which you can customize that position before launch. The default is the PBM's preference, not yours.

§ **Formulary change implementation timeline**
Know how quickly formulary changes can be implemented at your direction. The lag between an employer decision and a live formulary update is a measure of how much control your contract preserves.

§ **Process for adding prior authorization or coverage criteria**
Document the workflow, sign-off requirements, and timeline for adding PA or coverage criteria to a newly approved drug. This is where coverage gets shaped before utilization scales.